

An Act

ENROLLED HOUSE
BILL NO. 1687

By: Roe of the House

and

Rosino of the Senate

An Act relating to advance health care directives; creating the Uniform Health Care Decisions Act of 2026; providing short title; defining terms; establishing requirements to show capacity; providing for presumption of capacity under certain circumstances; providing for rebuttal of certain presumption; requiring certain notice; establishing procedures to object to certain finding; authorizing judicial review of certain finding; authorizing creation of health care instruction; establishing requirements for health care instruction; establishing requirements for creation of power of attorney for health care; authorizing creation of advance mental health care directive; establishing requirements for advance mental health care directive; establishing requirements for conflicting health care directives; providing optional form for advance health care directive; authorizing default surrogate who is a member of certain classes; establishing requirements for default surrogate; establishing procedures for disagreement of default surrogates; providing for disqualification of certain persons for acting as default surrogate; authorizing revocation of certain appointment, designation, or instruction; establishing requirements for validity of directives created out of state; directing resolution of conflicts of law; establishing duties of agent or default surrogate; providing for commencement and ceasing of powers of agent or default surrogate; prohibiting certain actions by agent or default surrogate; authorizing appointment of coagents and alternate agents; prohibiting authorization of certain persons to serve as agent or default surrogate; establishing certain duties for

health care professionals and health care institutions; defining term; authorizing certain decisions by guardians; providing immunity from civil or criminal liability for certain health care professionals or institutions; prohibiting certain conduct; providing for award of damages; authorizing issuance of certain court orders upon petition; construing provisions; requiring court to consider uniformity of law under certain circumstances; providing for validity of certain advance directives; clarifying applicability of act; requiring issuance of court order upon certain motion; repealing 63 O.S. 2021, Sections 3101.1, 3101.2, 3101.3, 3101.4, 3101.5, 3101.6, 3101.7, 3101.8, 3101.9, 3101.10, 3101.11, as amended by Section 737, Chapter 486, O.S.L. 2025, 3101.12, 3101.13, 3101.14, 3101.15, 3101.16, 3102.4, as amended by Section 15, Chapter 136, O.S.L. 2022, and 3102.5 (63 O.S. Supp. 2025, Sections 3101.11 and 3102.4), which relate to the Oklahoma Advance Directive Act; repealing Sections 1, 2, 3, as amended by Section 1, Chapter 154, O.S.L. 2023, 4, 5, as amended by Section 2, Chapter 154, O.S.L. 2023, 6, 7, 8, 9, 10, 11, 12, and 13, Chapter 136, O.S.L. 2022 (63 O.S. Supp. 2025, Sections 3111.1, 3111.2, 3111.3, 3111.4, 3111.5, 3111.6, 3111.7, 3111.8, 3111.9, 3111.10, 3111.11, 3111.12, and 3111.13), which relate to the Oklahoma Health Care Agent Act; providing for codification; and providing an effective date.

SUBJECT: Advance health care directives

BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

SECTION 1. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.1 of Title 63, unless there is created a duplication in numbering, reads as follows:

This act shall be known and may be cited as the "Uniform Health Care Decisions Act of 2026".

SECTION 2. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.2 of Title 63, unless there is created a duplication in numbering, reads as follows:

As used in this act:

1. "Advance health care directive" means a power of attorney for health care, health care instruction, or both. The term includes an advance mental health care directive;

2. "Advance mental health care directive" means a power of attorney for health care, health care instruction, or both, created under Section 9 of this act;

3. "Agent" means an individual appointed under a power of attorney for health care to make a health care decision for the individual who made the appointment. The term includes a coagent or alternate agent appointed under Section 20 of this act;

4. "Capacity" means having capacity under Section 3 of this act;

5. "Cohabitant" means each of two individuals who have been living together as a couple for at least one (1) year after each became an adult or was emancipated and who are not married to one another;

6. "Default surrogate" means an individual authorized under Section 12 of this act to make a health care decision for another individual;

7. "Electronic" means relating to technology having electrical, digital, magnetic, wireless, optical, electromagnetic, or similar capabilities;

8. "Family member" means a spouse, domestic partner, adult child, parent, or grandparent, or an adult descendant of a spouse, child, parent, or grandparent;

9. "Guardian" means a person appointed under law by a court to make decisions regarding the personal affairs of an individual, which may include health care decisions. The term does not include a guardian ad litem;

10. "Health care" means care or treatment or a service or procedure to maintain, monitor, diagnose, or otherwise affect an individual's physical or mental illness, injury, or condition. The term includes mental health care;

11. "Health care decision" means a decision made by an individual or the individual's surrogate regarding the individual's health care, including:

- a. selection or discharge of a health care professional or health care institution,
- b. approval or disapproval of a diagnostic test, surgical procedure, medication, therapeutic intervention, or other health care,
- c. direction to provide, withhold, or withdraw mechanical ventilation or other health care, and
- d. direction to provide, withhold, or withdraw artificially administered hydration or nutrition, subject to the limitations of the Hydration and Nutrition for Incompetent Patients Act;

12. "Health care institution" means a facility or agency licensed, certified, or otherwise authorized or permitted by other law to provide health care in this state in the ordinary course of business;

13. "Health care instruction" means a direction, whether or not in a record, made by an individual that indicates the individual's goals, preferences, or wishes concerning the provision, withholding, or withdrawal of health care. The term includes a direction intended to be effective if a specified condition arises;

14. "Health care professional" means a physician or other individual licensed, certified, or otherwise authorized or permitted by other law of this state to provide health care in this state in the ordinary course of business or the practice of the physician's or individual's profession;

15. "Individual" means an adult or emancipated minor;

16. "Life-sustaining care" and "life-sustaining treatment" mean health care that, in reasonable medical judgment, will be most

likely to be effective in ameliorating an individual's life-threatening conditions;

17. "Mental health care" means care or treatment or a service or procedure to maintain, monitor, diagnose, or otherwise affect an individual's mental illness or other psychiatric, psychological, or psychosocial condition;

18. "Nursing home" means a nursing facility as defined in Section 1919(a)(1) of the Social Security Act, 42 U.S.C., Section 1396r(a)(1), as amended, or skilled nursing facility as defined in Section 1819(a)(1) of the Social Security Act, 42 U.S.C., Section 1395i-3(a)(1), as amended;

19. "Person" means an individual, estate, business or nonprofit entity, government or governmental subdivision, agency, or instrumentality, or other legal entity;

20. "Person interested in the welfare of the individual" means:

- a. the individual's surrogate,
- b. a family member of the individual,
- c. the cohabitant of the individual,
- d. a public entity providing health care case management or protective services to the individual,
- e. a person appointed under other law to make decisions for the individual under a power of attorney for finances, or
- f. a person that has an ongoing personal or professional relationship with the individual, including a person that has provided educational or health care services or supported decision making to the individual;

21. "Physician" means an individual authorized to practice medicine from the State Board of Medical Licensure and Supervision or the State Board of Osteopathic Examiners;

22. "Power of attorney for health care" means a record in which an individual appoints an agent to make health care decisions for the individual;

23. "Reasonable medical judgment" means a health care judgment that would be made by a reasonably prudent physician knowledgeable about the medical conditions involved and the treatment possibilities with respect to such conditions;

24. "Reasonably available" means being able to be contacted without undue effort and being willing and able to act in a timely manner considering the urgency of an individual's health care situation. When used to refer to an agent or default surrogate, the term includes being willing and able to comply with the duties under Section 17 of this act in a timely manner considering the urgency of an individual's health care situation;

25. "Record" means information:

- a. inscribed on a tangible medium, or
- b. stored in an electronic or other medium and retrievable in perceivable form;

26. "Responsible health care professional" means:

- a. a health care professional designated by an individual or the individual's surrogate to have primary responsibility for the individual's health care or for overseeing a course of treatment, or
- b. in the absence of a designation under subparagraph a of this paragraph, or if the professional designated under subparagraph a of this paragraph is not reasonably available, a health care professional who has primary responsibility for overseeing the individual's health care or for overseeing a course of treatment;

27. "Sign" means with present intent to authenticate or adopt a record:

- a. executing or adopting a tangible symbol, or
- b. attaching to or logically associating with the record an electronic symbol, sound, or process;

28. "State" means a state of the United States, the District of Columbia, the Commonwealth of Puerto Rico, the United States Virgin Islands, or any other territory, or possession subject to the jurisdiction of the United States. The term includes a federally recognized Indian tribe;

29. "Supported decision making" means assistance, from one or more persons of an individual's choosing, that helps the individual make or communicate a decision, including by helping the individual understand the nature and consequences of the decision; and

30. "Surrogate" means:

- a. an agent,
- b. a default surrogate, or
- c. a guardian authorized to make health care decisions.

SECTION 3. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.3 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. An individual has capacity for the purpose of this act if the individual:

1. Is willing and able to communicate a decision independently or with appropriate services, technological assistance, supported decision making, or other reasonable accommodation; and

2. In making or revoking:

- a. a health care decision, understands the nature and consequences of the decision, including the primary risks and benefits of the decision,
- b. a health care instruction, understands the nature and consequences of the instruction, including the primary risks and benefits of the choices expressed in the instruction, and
- c. an appointment of an agent under a health care power of attorney or identification of a default surrogate under paragraph 1 of subsection B of Section 12 of this act, recognizes the identity of the individual

being appointed or identified and understands the general nature of the relationship of the individual making the appointment or identification with the individual being appointed or identified.

B. The right of an individual who has capacity to make a decision about the individual's health care is not affected by whether the individual creates or revokes an advance health care directive.

SECTION 4. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.4 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. An individual is presumed to have capacity to make or revoke a health care decision, health care instruction, and power of attorney for health care unless:

1. A court has found the individual lacks capacity to do so; or
2. The presumption is rebutted under subsection B of this section.

B. Subject to Sections 5 and 6 of this act, a presumption under subsection A of this section may be rebutted by a finding that the individual lacks capacity:

1. Subject to subsection C of this section, made on the basis of a contemporaneous examination by any of the following:

- a. a physician,
- b. a psychologist licensed or otherwise authorized to practice in this state,
- c. an individual with training and expertise in the finding of lack of capacity who is licensed or otherwise authorized to practice in this state as an Advanced Practice Registered Nurse, a physician assistant, or a social worker, or
- d. a responsible health care professional not described in subparagraph a, b, or c of this paragraph if:

- (1) the individual about whom the finding is to be made is experiencing a health condition requiring a decision regarding health care treatment to be made promptly to avoid loss of life or serious harm to the health of the individual, and
- (2) an individual listed in subparagraph a, b, or c of this paragraph is not reasonably available;

2. Made in accordance with accepted standards of the profession and the scope of practice of the individual making the finding and to a reasonable degree of certainty; and

3. Documented in a record signed by the individual making the finding that includes an opinion of the cause, nature, extent, and probable duration of the lack of capacity.

C. The finding under subsection B of this section may not be made by:

1. A family member of the individual presumed to have capacity;

2. The cohabitant of the individual or a descendant of the cohabitant; or

3. The individual's surrogate, a family member of the surrogate, or a descendant of the surrogate.

D. If the finding under subsection B of this section was based on a condition the individual no longer has or a responsible health care professional subsequently has good cause to believe the individual has capacity, the individual is presumed to have capacity unless a court finds the individual lacks capacity or the presumption is rebutted under subsection B of this section.

SECTION 5. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.5 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. As soon as reasonably feasible, an individual who makes a finding under subsection B of Section 4 of this act shall inform the individual about whom the finding was made or the individual's responsible health care professional of the finding.

B. As soon as reasonably feasible, a responsible health care professional who is informed of a finding under subsection B of Section 4 of this act shall inform the individual about whom the finding was made and the individual's surrogate.

C. An individual found under subsection B of Section 4 of this act to lack capacity may object to the finding:

1. By orally informing a responsible health care professional;
2. In a record provided to a responsible health care professional or the health care institution in which the individual resides or is receiving care; or
3. By another act that clearly indicates the individual's objection.

D. If the individual objects under subsection C of this section, the finding under subsection B of Section 4 of this act is not sufficient to rebut a presumption of capacity under subsection A of Section 4 of this act and the individual shall be treated as having capacity unless:

1. The individual withdraws the objection;
2. A court finds the individual lacks the presumed capacity;
3. The individual is experiencing a health condition requiring a decision regarding health care treatment to be made promptly to avoid imminent loss of life or serious harm to the health of the individual; or
4. Subject to subsection B of this section, the finding is confirmed by a second finding made by an individual authorized under paragraph 1 of subsection B of Section 4 of this act who:
 - a. did not make the first finding,
 - b. is not a family member of the individual who made the first finding, and
 - c. is not the cohabitant of the individual who made the first finding or a descendant of the cohabitant.

E. Neither a finding in the circumstances described in paragraph 3 of subsection D of this section nor a second finding that the individual lacks capacity under paragraph 4 of subsection D of this section shall be sufficient to rebut the presumption of capacity if the individual is requesting the provision or continuation of life-sustaining treatment and the finding is being used to make a decision to withhold or withdraw the treatment.

F. As soon as reasonably feasible, a health care professional who is informed of an objection under subsection C of this section shall:

1. Communicate the objection to a responsible health care professional; and

2. Document the objection and the date of the objection in the individual's medical record or communicate the objection and the date of the objection to an administrator with responsibility for medical records of the health care institution providing health care to the individual, who shall document the objection and the date of the objection in the individual's medical record.

SECTION 6. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.6 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. An individual found under subsection B of Section 4 of this act to lack capacity, a responsible health care professional, the health care institution providing health care to the individual, or a person interested in the welfare of the individual may petition the court in the respective county in which the individual resides or is located to determine whether the individual lacks capacity.

B. The court in which a petition under subsection A of this section is filed shall appoint legal counsel to represent the individual if the individual does not have legal counsel. The court shall hear the petition as soon as possible, but not later than seven (7) days after the petition is filed. As soon as possible, but not later than seven (7) days after the hearing, the court shall determine whether the individual lacks capacity. The court may determine that the individual lacks capacity only if the court finds by clear and convincing evidence that the individual lacks capacity.

SECTION 7. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.7 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. An individual may create a health care instruction that expresses the individual's preferences for future health care, including preferences regarding:

1. Health care professionals or health care institutions;
2. How a health care decision will be made and communicated;
3. Persons that should or should not be consulted regarding a health care decision;
4. A person to serve as guardian for the individual if one is appointed; and
5. An individual to serve as a default surrogate.

B. A health care professional to whom an individual communicates or provides an instruction under subsection A of this section shall document the instruction and the date of the instruction in the individual's medical record or communicate the instruction and date of the instruction to an administrator with responsibility for medical records of the health care institution providing health care to the individual, who shall document the instruction and the date of the instruction in the individual's medical record.

C. A health care instruction made by an individual that conflicts with an earlier health care instruction made by the individual, including an instruction documented in a medical order, revokes the earlier instruction to the extent of the conflict.

D. A health care instruction may be in the same record as a power of attorney for health care.

SECTION 8. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.8 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. An individual may create a power of attorney for health care to appoint an agent to make health care decisions for the individual.

B. An individual shall be disqualified from acting as agent for an individual who lacks capacity to make health care decisions if:

1. A court finds that the potential agent poses a danger to the individual's well-being, even if the court does not issue a restraining order against the potential agent; or

2. The potential agent is an owner, operator, employee, or contractor of a nursing home, or other residential care facility, in which the individual resides or is receiving care, unless the owner, operator, employee, or contractor is a family member of the individual, the cohabitant of the individual, or a descendant of the cohabitant.

C. A health care decision made by an agent is effective without judicial approval.

D. A power of attorney for health care shall be in a record, signed by the individual creating the power, and signed by an adult witness who:

1. Reasonably believes the act of the individual to create the power of attorney is voluntary and knowing; and

2. Is not:

a. the agent appointed by the individual,

b. the agent's spouse or cohabitant, or

c. if the individual resides or is receiving care in a nursing home, or other residential care facility, the owner, operator, employee, or contractor of the nursing home or other residential care facility; and

3. Is present when the individual signs the power of attorney or when the individual represents that the power of attorney reflects the individual's wishes.

E. A witness under subsection D of this section is considered present if the witness and the individual are:

1. Physically present in the same location;

2. Using electronic means that allow for real time audio and visual transmission and communication in real time to the same extent as if the witness and the individual were physically present in the same location; or

3. Able to speak to and hear each other in real time through audio connection if:

- a. the identity of the individual is personally known to the witness, or
- b. the witness is able to authenticate the identity of the individual by receiving accurate answers from the individual that enables the authentication.

F. A power of attorney for health care may include a health care instruction.

SECTION 9. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.9 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. An individual may create an advance health care directive that addresses only mental health care for the individual. The advance mental health care directive may include a health care instruction, a power of attorney for health care, or both.

B. A health care instruction under this section may include the individual's:

1. General philosophy and objectives regarding mental health care; and

2. Specific goals, preferences, and wishes regarding the provision, withholding, or withdrawal of a form of mental health care, including:

- a. preferences regarding professionals, programs, and facilities,
- b. admission to a mental health facility, including duration of admission,
- c. preferences regarding medications,

- d. refusal to accept a specific type of mental health care, including a medication, and
- e. preferences regarding crisis intervention.

C. A power of attorney for health care under this section may appoint an agent to make decisions only for mental health care.

D. An individual may direct in an advance mental health care directive that, if the individual is experiencing a psychiatric or psychological event specified in the directive, the individual may not revoke the directive or a part of the directive.

E. If an advance mental health care directive includes a direction under subsection D of this section, the advance mental health care directive shall be in a record that is separate from any other advance health care directive created by the individual and signed by the individual creating the advance mental health care directive and at least two adult witnesses who:

1. Attest that to the best of their knowledge the individual:

- a. understood the nature and consequences of the direction, including its risks and benefits, and
- b. made the direction voluntarily and without coercion or undue influence;

2. Are not:

- a. the agent appointed by the individual,
- b. the agent's spouse, or cohabitant, and
- c. if the individual resides in a nursing home, or other residential care facility, the owner, operator, employee, or contractor of the nursing home or other residential care facility; and

3. Are physically present in the same location as the individual.

SECTION 10. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.10 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. If a direction in an advance mental health care directive of an individual conflicts with a direction in another advance health care directive of the individual, the later direction revokes the earlier direction to the extent of the conflict.

B. An appointment of an agent to make decisions only for mental health care for an individual does not revoke an earlier appointment of an agent to make other health care decisions for the individual. A later appointment revokes the authority of an agent under the earlier appointment to make decisions about mental health care unless otherwise specified in the power of attorney making the later appointment.

C. An appointment of an agent to make health care decisions for an individual other than decisions about mental health care made after appointment of an agent authorized to make only mental health care decisions does not revoke the appointment of the agent authorized to make only mental health care decisions.

SECTION 11. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.11 of Title 63, unless there is created a duplication in numbering, reads as follows:

The following form may be used to create an advance health care directive:

ADVANCE HEALTH CARE DIRECTIVE

HOW YOU CAN USE THIS FORM

You can use this form if you wish to name someone to make health care decisions for you in case you cannot make decisions for yourself. This is called giving the person a power of attorney for health care. This person is called your agent.

You can also use this form to state your wishes, preferences, and goals for health care, and to say if you want to be an organ donor after you die.

YOUR NAME AND DATE OF BIRTH

Name:

Date of birth:

PART A: NAMING AN AGENT

This part lets you name someone else to make health care decisions for you. You may leave any item blank.

1. NAMING AN AGENT

I want the following person to make health care decisions for me if I cannot make decisions for myself:

Name:

Optional contact information (it is helpful to include information such as address, phone, and email):

2. NAMING AN ALTERNATE AGENT

I want the following person to make health care decisions for me if I cannot and my agent is not able or available to make them for me:

Name:

Optional contact information (it is helpful to include information such as address, phone, and email):

3. LIMITING YOUR AGENT'S AUTHORITY

I give my agent the power to make all health care decisions for me if I cannot make those decisions for myself, except the following:

(If you do not add a limitation here, your agent will be able to make all health care decisions that an agent is permitted to make under state law. State law does not permit your agent to withhold or withdraw food and liquids needed for you to stay alive except as you specify in this document or by any other means the law provides.)

PART B: HEALTH CARE INSTRUCTIONS

This part lets you state your priorities for health care and to state types of health care you do and do not want.

1. INSTRUCTIONS ABOUT LIFE-SUSTAINING TREATMENT

This section gives you the opportunity to say how you want your agent to act while making decisions for you. You may mark or initial each choice. You also may leave any choice blank.

Treatment. Medical treatment needed to keep me alive but not needed for comfort or any other purpose should (mark or initial all that apply):

(____) Always be given to me. (If you mark or initial this choice, you should not mark or initial other choices in this "treatment" section.)

(____) Not be given to me if I have a condition that is not curable and is expected to cause me to die within two weeks, even if treated.

(____) Not be given to me if I am unconscious and I am not expected to be conscious again.

(____) Not be given to me if I have a medical condition from which I am not expected to recover that prevents me from communicating with people I care about, caring for myself, and recognizing family and friends.

(____) Other (write what you want or do not want):

Food and liquids. If I can't swallow and staying alive requires me to get food or liquids through a tube or other means for the rest of my life, then food or liquids should (mark or initial all that apply):

(____) Always be given to me. (If you mark or initial this choice, you should not mark or initial other choices in this "food and liquids" section.)

(____) Not be given to me if I have a condition that is not curable and is expected to cause me to die within two weeks, even if treated.

(____) Not be given to me if I am unconscious and am not expected to be conscious again.

(____) Not be given to me if I have a medical condition from which I am not expected to recover that prevents me from communicating with people I care about, caring for myself, and recognizing family and friends.

(____) Other (write what you want or do not want):

Pain relief. If I am in significant pain, care that will keep me comfortable but is likely to shorten my life should (mark or initial all that apply):

(____) Always be given to me. (If you mark or initial this choice, you should not mark or initial other choices in this "pain relief" section.)

(____) Never be given to me. (If you mark or initial this choice, you should not mark or initial other choices in this "pain relief" section.)

(____) Be given to me if I have a condition that is not curable and is expected to cause me to die soon, even if treated.

(____) Be given to me if I am unconscious and am not expected to be conscious again.

(____) Be given to me if I have a medical condition from which I am not expected to recover that prevents me from communicating with people I care about, caring for myself, and recognizing family and friends.

(____) Other (write what you want or do not want):

2. MY PRIORITIES

You can use this section to indicate what is important to you, and what is not important to you. This information can help your agent make decisions for you if you cannot. It also helps others understand your preferences.

You may leave any choice blank.

My priorities:

Having my agent talk with my family before making decisions about my care is:

(____) Very important

(____) Somewhat important

(____) Not important

Having my agent talk with my friends before making decisions about my care is:

(____) Very important

(____) Somewhat important

(____) Not important

3. OTHER INSTRUCTIONS

You can write in this section more information about your goals, values, and preferences for treatment, including care you want or do not want. You can also use this section to name anyone who you do not want to make decisions for you under any conditions.

PART C: OPTIONAL SPECIAL POWERS AND GUIDANCE

This part lets you give your agent additional powers, and to provide more guidance about your wishes. You may mark or initial each choice. You also may leave any choice blank.

1. OPTIONAL SPECIAL POWERS

My agent can do the following things ONLY if I have marked or initialed them below:

(____) Consent to withholding of food or liquid provided through a tube or by other means and required to keep me alive. (If I do not mark or initial this choice, my agent MAY NOT prevent me from receiving such food or liquid.)

(____) Admit me as a voluntary patient to a facility for mental health treatment for up to _____ days (write in the number of days you want like 7, 14, 30, or another number). (If I do not mark or initial this choice, my agent MAY NOT admit me as a voluntary patient to this type of facility.)

(____) Place me in a nursing home, other than a short-term placement for rehabilitation following hospitalization, even if my needs can be met somewhere else. (If I do not mark or initial this choice, my agent MAY NOT do this.)

2. ACCESS TO MY HEALTH INFORMATION

My agent may obtain, examine, and share information about my health needs and health care if I am not able to make decisions for myself. If I mark or initial below, my agent may also do that at any time my agent thinks it will help me.

(____) I give my agent permission to obtain, examine, and share information about my health needs and health care whenever my agent thinks it will help me.

3. FLEXIBILITY FOR MY AGENT

Mark or initial below if you want to give your agent flexibility in following instructions you provide in this form. If you do not, your agent must follow the instructions even if your agent thinks something else would be better for you.

(____) I give my agent permission to be flexible in applying these instructions if my agent thinks it would be in my best interest based on what my agent knows about me.

4. NOMINATION OF GUARDIAN

You can say who you would want as your guardian if you needed one. A guardian is a person appointed by a court to make decisions for someone who cannot make decisions. Filling this out does NOT mean you want or need a guardian.

If a court appoints a guardian to make personal decisions for me, I want the court to choose:

(____) My agent named in this form. If my agent cannot be a guardian, I want the alternate agent named in this form.

(____) Other (write who you would want and their contact information):

PART D: ORGAN DONATION

This part lets you donate your organs after you die. You may leave any item blank. If you leave both choices under "DONATION" blank, your health care agent may decide whether your organs will be donated.

1. DONATION

You may mark or initial only one choice.

(____) I donate my organs, tissues, and other body parts after I die, even if it requires maintaining treatments that conflict with other instructions I have put in this form, EXCEPT for those I list below (list any body parts you do NOT want to donate):

(____) I do not want my organs, tissues, or body parts donated to anybody for any reason. (If you mark or initial this choice, you should skip the "PURPOSE OF DONATION" section.)

2. PURPOSE OF DONATION

You may mark or initial all that apply. (If you do not mark or initial any of the purposes below, your donation can be used for all of them.)

(____) Transplant

(____) Therapy

(____) Research

(____) Education

(____) All of the above

PART E: SIGNATURES

YOUR SIGNATURE

Sign your name:

Today's date:

City/Town/Village and State (optional):

SIGNATURE OF A WITNESS

You need a witness if you are using this form to name an agent. The witness must be an adult and cannot be the person you are naming as agent or the agent's spouse, or someone the agent lives with as a couple. If you live or are receiving care in a nursing home, the witness cannot be an employee or contractor of the home or someone who owns or runs the home.

Name of Witness:

Signature of Witness:

(Only sign as a witness if you think the person signing above is doing it voluntarily.)

Date witness signed:

PART F: INFORMATION FOR AGENTS

1. If this form names you as an agent, you can make decisions about health care for the person who named you when the person cannot make their own.

2. If you make a decision for the person, follow any instructions the person gave, including any in this form.

3. If you do not know what the person would want, make the decision that you think is in the person's best interest. To figure out what is in the person's best interest, consider the person's values, preferences, and goals if you know them or can learn them. Some of these preferences may be in this form. You should also consider any behavior or communication from the person that indicates what the person currently wants.

4. If this form names you as an agent, you can also get and share the person's health information. Unless the person has said so in this form, you can get or share this information only when the person cannot make decisions about the person's health care.

SECTION 12. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.12 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. A default surrogate may make a health care decision for an individual who lacks capacity to make health care decisions and for whom an agent, or guardian authorized to make health care decisions, has not been appointed or is not reasonably available.

B. Unless the individual has an advance health care directive that indicates otherwise, a member of the following classes, in descending order of priority, who is reasonably available and not disqualified under Section 14 of this act, may act as a default surrogate for the individual:

1. An adult the individual has identified, other than in a power of attorney for health care, to make a health care decision for the individual if the individual cannot make the decision;

2. The individual's spouse, unless:

a. a petition for annulment, divorce, dissolution of marriage, legal separation, or termination has been filed and not dismissed or withdrawn,

b. a decree of annulment, divorce, dissolution of marriage, legal separation, or termination has been issued,

c. the individual and the spouse have agreed in a record to a legal separation, or

d. the spouse has abandoned the individual for more than one (1) year;

3. The individual's adult child or parent;

4. The individual's cohabitant;

5. The individual's adult sibling;

6. The individual's adult grandchild or grandparent;

7. An adult not listed in paragraphs 1 through 6 of this subsection who has assisted the individual with supported decision making routinely during the preceding six (6) months;

8. The individual's adult stepchild not listed in paragraphs 1 through 7 of this subsection whom the individual actively parented during the stepchild's minor years and with whom the individual has an ongoing relationship; or

9. An adult not listed in paragraphs 1 through 8 of this subsection who has exhibited special care and concern for the individual and is familiar with the individual's personal values.

C. A responsible health care professional may require an individual who assumes authority to act as a default surrogate to provide a declaration in a record under penalty of perjury stating facts and circumstances reasonably sufficient to establish the authority.

D. If a responsible health care professional reasonably determines that an individual who assumed authority to act as a default surrogate is not willing or able to comply with a duty under Section 17 of this act or fails to comply with the duty in a timely manner, the professional may recognize the individual next in priority under subsection B of this section as the default surrogate.

E. A health care decision made by a default surrogate is effective without judicial approval.

F. Prior to making a health care decision for an individual pursuant to subsection A of this section, a responsible health care professional may require a default surrogate to provide to the responsible health care professional a signed copy of the following statement to be entered into the patient's medical record:

"I hereby certify that:

I have not been convicted of, pleaded guilty to, or pleaded no contest to the crimes of abuse, verbal abuse, neglect, or financial exploitation by a caregiver; exploitation of an elderly person or

disabled adult; or abuse, neglect, exploitation, or sexual abuse of a child;

I have not been found to have committed abuse, verbal abuse, or exploitation by a final investigative finding of the State Department of Health or the Department of Human Services or by a finding of an administrative law judge, unless it was overturned on appeal; and I have not been criminally charged as a person responsible for the care of a vulnerable adult with a crime resulting in the death or near death of a vulnerable adult."

SECTION 13. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.13 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. A default surrogate who assumes authority under Section 12 of this act shall inform a responsible health care professional if two or more members of a class under subsection B of Section 12 of this act have assumed authority to act as default surrogates and the members do not agree on a health care decision.

B. A responsible health care professional shall comply with the decision of a majority of the members of the class with highest priority under subsection B of Section 12 of this act who have communicated their views to the professional and who the professional reasonably believes are acting consistently with their duties under Section 17 of this act.

C. If a responsible health care professional is informed that the members of the class who have communicated their views to the professional are evenly divided concerning the health care decision, the professional shall make a reasonable effort to solicit the views of members of the class who are reasonably available but have not yet communicated their views to the professional. The professional, after the solicitation, shall comply with the decision of a majority of the members who have communicated their views to the professional and who the professional reasonably believes are acting consistently with their duties under Section 17 of this act.

D. If the class remains evenly divided after the effort is made under subsection C of this section, the health care decision shall be made as provided by other laws of this state regarding the treatment of an individual who is found to lack capacity.

E. Nothing in this section shall be construed to supersede or limit the remedies available under Section 27 of this act.

SECTION 14. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.14 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. An individual for whom a health care decision would be made may disqualify another individual from acting as default surrogate for the first individual. The disqualification shall be in a record signed by the first individual or communicated verbally or nonverbally to the individual being disqualified, another individual, or a responsible health care professional. Disqualification under this subsection shall be effective even if made by an individual who lacks capacity to make an advance directive if the individual clearly communicates a desire that the individual being disqualified cannot make health care decisions for the individual.

B. An individual is disqualified from acting as a default surrogate for an individual who lacks capacity to make health care decisions if:

1. A court finds that the potential default surrogate poses a danger to the individual's well-being, even if the court does not issue a restraining order against the potential surrogate;

2. The potential default surrogate is an owner, operator, employee, or contractor of a nursing home, or other residential care facility, in which the individual is residing or receiving care unless the owner, operator, employee, or contractor is a family member of the individual, the cohabitant of the individual, or a descendant of the cohabitant; or

3. The potential default surrogate refuses to provide a timely declaration under subsection C or F of Section 12 of this act.

SECTION 15. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.15 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. An individual may revoke the appointment of an agent, the designation of a default surrogate, or a health care instruction in whole or in part, unless:

1. A court finds the individual lacks capacity to do so;

2. The individual is found under subsection B of Section 4 of this act to lack capacity to do so, and if the individual objects to the finding, the finding is confirmed under paragraph 4 of subsection D of Section 5 of this act and is not revoked under Section 6 of this act; or

3. The individual created an advance mental health care directive that includes the provision under subsection D of Section 9 of this act, and the individual is experiencing the psychiatric or psychological event specified in the directive.

B. Revocation under subsection A of this section may be by any act of the individual that clearly indicates that the individual intends to revoke the appointment, designation, or instruction, including an oral statement to a health care professional.

C. Except as provided in Section 10 of this act, an advance health care directive of an individual that conflicts with another advance health care directive of the individual revokes the earlier directive to the extent of the conflict.

D. Unless otherwise provided in an individual's advance health care directive appointing an agent, the appointment of a spouse of an individual as agent for the individual is revoked if:

1. A petition for annulment, divorce, dissolution of marriage, legal separation, or termination has been filed and not dismissed or withdrawn;

2. A decree of annulment, divorce, dissolution of marriage, legal separation, or termination has been issued;

3. The individual and the spouse have agreed in a record to a legal separation; or

4. The spouse has abandoned the individual for more than one (1) year.

SECTION 16. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.16 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. Subject to subsection C of Section 19 of this act, an advance health care directive created outside this state is valid if it complies with:

1. The law of the state specified in the directive or, if a state is not specified, the state in which the individual created the directive; or

2. The provisions of this act.

B. A person may assume without inquiry that an advance health care directive is genuine, valid, and still in effect, and may implement and rely on it, unless the person has good cause to believe the directive is invalid or has been revoked.

C. An advance health care directive, revocation of a directive, or a signature on a directive or revocation may not be denied legal effect or enforceability solely because it is in electronic form.

D. Evidence relating to an advance health care directive, revocation of a directive, or a signature on a directive or revocation may not be excluded in a proceeding solely because the evidence is in electronic form.

E. This act shall not affect the validity of an electronic record or signature that is valid under Section 15-101 et seq. of Title 12A of the Oklahoma Statutes.

F. Except as provided in subsections D and E of Section 19, Section 21, subsection H of Section 22, and subsections A and D of Section 28 of this act, if this act conflicts with any other law of this state relating to the creation, execution, implementation, or revocation of an advance health care directive, the provisions of this act shall prevail.

SECTION 17. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.17 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. An agent or default surrogate has a fiduciary duty to the individual for whom the agent or default surrogate is acting when exercising or purporting to exercise a power under Section 18 of this act.

B. An agent or default surrogate shall make a health care decision in accordance with the direction of the individual in an advance health care directive and other goals, preferences, and wishes of the individual to the extent known or reasonably ascertainable by the agent or default surrogate.

C. If there is not a direction in an advance health care directive and the goals, preferences, and wishes of the individual regarding a health care decision are not known or reasonably ascertainable by the agent or default surrogate, the agent or default surrogate shall make the decision in accordance with the agent's or default surrogate's reasonable determination of the individual's best interest.

D. In determining the individual's best interest under subsection C of this section, the agent or default surrogate shall:

1. Give primary consideration to the individual's contemporaneous communications, including verbal and nonverbal expressions;

2. Consider the individual's values to the extent known or reasonably ascertainable by the agent or default surrogate; and

3. Consider the risks and benefits of the potential health care decision.

E. As soon as reasonably feasible, an agent or default surrogate who is informed of a revocation of an advance health care directive or disqualification of the agent or default surrogate shall communicate the revocation or disqualification to a responsible health care professional.

SECTION 18. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.18 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. Except as provided in subsection C of this section, the power of an agent or default surrogate commences when the individual is found under subsection B of Section 4 of this act or by a court to lack capacity to make a health care decision. The power shall cease if the individual later is found to have capacity to make a health care decision, or the individual objects under subsection C of Section 5 of this act to the finding of lack of capacity under

subsection B of Section 4 of this act. Subject to subsection E of Section 5 of this act, the power shall resume if:

1. The power ceased because the individual objected under subsection C of Section 5 of this act; and

2. The finding of lack of capacity is confirmed under paragraph 4 of subsection D of Section 5 of this act, or a court finds that the individual lacks capacity to make a health care decision.

B. An agent or default surrogate may request, receive, examine, copy, and consent to the disclosure of medical and other health care information about the individual if the individual would have the right to request, receive, examine, copy, or consent to the disclosure of the information.

C. A power of attorney for health care may provide that the power of an agent under subsection B of this section commences on appointment.

D. If no other person is authorized to do so, an agent or default surrogate may apply for public or private health insurance and benefits on behalf of the individual. An agent or default surrogate who may apply for insurance and benefits does not, solely by reason of the power, have a duty to apply for the insurance or benefits.

E. An agent or default surrogate may not consent to voluntary admission of the individual to a facility for mental health treatment unless:

1. Voluntary admission is specifically authorized by the individual in an advance health care directive in a record; and

2. The admission is for no more than the maximum of the number of days specified in the directive.

F. Except as provided in subsection G of this section, an agent or default surrogate may not consent to placement of the individual in a nursing home, other than for short-term rehabilitation services following hospitalization, if the individual objects to the placement and a reasonable alternative living arrangement is feasible.

G. If specifically authorized by the individual in an advance health care directive in a record, an agent or default surrogate may consent to placement of the individual in a nursing home even if a reasonable alternative living arrangement is feasible.

SECTION 19. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.19 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. If an individual has a long-term disability requiring routine treatment by mechanical ventilation and a history of using the treatment without objection, an agent or default surrogate may not consent to withhold or withdraw the treatment unless:

1. The treatment is both not necessary to sustain the individual's life and not necessary to maintain the individual's health or well-being;

2. The individual has expressly authorized the withholding or withdrawal in a health care instruction that has not been revoked; or

3. The individual has experienced a life-threatening condition that is not curable and is expected to cause the individual to die within two (2) weeks, even if treated, and the individual has not:

- a. given a direction inconsistent with withholding or withdrawal, or
- b. communicated by verbal or nonverbal expression a desire for mechanical ventilation.

B. An advance directive that is executed in this state shall not be deemed to authorize the withholding or withdrawal of artificially administered nutrition or hydration unless it specifically authorizes the withholding or withdrawal of artificially administered nutrition or hydration in the declarant's own words or by a separate section, separate paragraph, or other separate subdivision that deals only with nutrition or hydration and which section, paragraph, or other subdivision is separately initialed, separately signed, or otherwise separately marked by the declarant.

C. Execution of an advance directive by an individual that provides for the provision, withholding, or withdrawal of life-

sustaining treatment for the individual or for the appointment of another to give directions to provide, withhold, or withdraw life-sustaining treatment, executed in another state in compliance with the laws of that state or of this state, shall be valid for purposes of the this act to the extent the advance directive does not exceed authorizations allowed under the laws of this state. No such advance directive shall be deemed to authorize the withholding or withdrawal of artificially administered nutrition or hydration unless it specifically authorizes such withholding or withdrawal, and the advance directive either:

1. Was executed by a person who was not a resident of this state at the time of execution; or

2. Specifically authorizes the withholding or withdrawal of artificially administered nutrition or hydration in the declarant's own words or by a separate section, separate paragraph, or other separate subdivision that deals only with nutrition or hydration, and such section, paragraph, or other subdivision is separately initialed, separately signed, or otherwise separately marked by the person executing the advance directive.

D. Decisions by a default surrogate or agent to consent to withholding or withdrawal of nutrition or hydration are subject to the provisions of the Hydration and Nutrition for Incompetent Patients Act.

E. A default surrogate may not make a health care decision if, under other laws of this state, the decision:

1. May not be made by a guardian; or

2. May be made by a guardian only if the court appointing the guardian specifically authorizes the guardian to make the decision.

SECTION 20. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.20 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. An individual in a power of attorney for health care may appoint multiple individuals as coagents. Unless the power of attorney provides otherwise, each coagent may exercise independent authority.

B. An individual in a power of attorney for health care may appoint one or more individuals to act as alternate agents if a predecessor agent resigns, dies, becomes disqualified, is not reasonably available, or otherwise is unwilling or unable to act as agent.

C. Unless the power of attorney provides otherwise, an alternate agent has the same authority as the original agent:

1. At any time the original agent is not reasonably available or is otherwise unwilling or unable to act, for the duration of the unavailability, unwillingness, or inability to act; or

2. If the original agent and all other predecessor agents have resigned or died or are disqualified from acting as agent.

SECTION 21. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.21 of Title 63, unless there is created a duplication in numbering, reads as follows:

No person shall be authorized as a default surrogate to make health care decisions for a patient if that person:

1. Has been convicted of, pled guilty to, or pled no contest to any violation of Section 843.1, 843.2, 843.4, or 843.5 of Title 21 of the Oklahoma Statutes;

2. Has been found to have committed abuse, verbal abuse, or exploitation, as defined in Section 10-103 of Title 43A of the Oklahoma Statutes, by a final State Department of Health or Department of Human Services investigative finding, or by an administrative law judge finding, unless that finding has been overturned through judicial review; or

3. Has been criminally charged as described in subsection B of Section 10-110.1 of Title 43A of the Oklahoma Statutes, unless the person has been acquitted or those charges have been finally dismissed.

SECTION 22. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.22 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. A responsible health care professional who is aware that an individual has been found to lack capacity to make a decision shall

make a reasonable effort to determine if the individual has a surrogate.

B. If possible before implementing a health care decision made by a surrogate, a responsible health care professional, as soon as reasonably feasible, shall communicate to the individual the decision made and the identity of the surrogate.

C. A responsible health care professional who makes or is informed of a finding that an individual lacks capacity to make a health care decision or no longer lacks capacity, or that other circumstances exist that affect a health care instruction or the authority of a surrogate, as soon as reasonably feasible, shall:

1. Document the finding or circumstance in the individual's medical record; and

2. If possible, communicate to the individual and the individual's surrogate the finding or circumstance and that the individual may object under subsection C of Section 5 of this act to the finding under subsection B of Section 4 of this act.

D. A responsible health care professional who is informed that an individual has created or revoked an advance health care directive, or that a surrogate for an individual has been appointed, designated, or disqualified, shall:

1. Document the information as soon as reasonably feasible in the individual's medical record; and

2. If evidence of the directive, revocation, appointment, designation, or disqualification is in a record, request a copy and, upon receipt, cause the copy to be included in the individual's medical record.

E. Except as provided in subsections F and G of this section, a health care professional or health care institution providing health care to an individual shall comply with:

1. A health care instruction given by the individual regarding the individual's health care;

2. A reasonable interpretation by the individual's surrogate of an instruction given by the individual; and

3. A health care decision for the individual made by the individual's surrogate in accordance with Sections 17 and 18 of this act to the same extent as if the decision had been made by the individual at a time when the individual had capacity.

F. A health care professional or a health care institution may refuse to provide health care consistent with a health care instruction or health care decision if:

1. The instruction or decision is contrary to a policy of the health care institution providing care to the individual that is based expressly on reasons of conscience, and the policy was timely communicated to the individual or to the individual's surrogate;

2. The care would require health care that is not available to the professional or institution; or

3. Compliance with the instruction or decision would:

- a. require the professional to provide care that is contrary to the professional's religious belief or moral conviction if other law permits the professional to refuse to provide care for that reason,
- b. require the professional or institution to provide care that is contrary to generally accepted health care standards applicable to the professional or institution, or
- c. violate a court order or other law.

G. A health care professional or health care institution that refuses to provide care under subsection F of this section shall:

1. As soon as reasonably feasible, inform the individual, if possible, and the individual's surrogate of the refusal;

2. Immediately make a reasonable effort to transfer the individual to another health care professional or health care institution that is willing to comply with the instruction or decision; and

3. Either:

- a. if care is refused under paragraph 1 or 2 of subsection F of this section, provide life-sustaining care and care needed to keep or make the individual comfortable, consistent with accepted medical standards to the extent feasible, until a transfer is made, or
- b. if care is refused under paragraph 3 of subsection F of this section, provide life-sustaining care and care needed to keep or make the individual comfortable, consistent with accepted medical standards, until a transfer is made.

H. As used in this section, "generally accepted health care standards" and "accepted medical standards" shall not be interpreted to supersede, or to authorize any violation of, the Nondiscrimination in Treatment Act.

SECTION 23. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.23 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. A guardian may refuse to comply with or revoke the individual's advance health care directive only if the court appointing the guardian expressly orders the noncompliance or revocation.

B. Unless a court orders otherwise, a health care decision made by an agent appointed by an individual subject to guardianship prevails over a decision of the guardian appointed for the individual.

SECTION 24. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.24 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. A health care professional or health care institution acting in good faith shall not be subject to civil or criminal liability or to discipline for unprofessional conduct for:

1. Complying with a health care decision made for an individual by another person if compliance is based on a reasonable belief that the person has authority to make the decision, including a decision to withhold or withdraw health care;

2. Refusing to comply with a health care decision made for an individual by another person if the refusal is based on a reasonable belief that the person lacked authority or capacity to make the decision;

3. Complying with an advance health care directive based on a reasonable belief that the directive is valid;

4. Refusing to comply with an advance health care directive based on a reasonable belief that the directive is not valid, including a reasonable belief that the directive was not made by the individual or, after its creation, was substantively altered by a person other than the individual who created it;

5. Determining that an individual who otherwise might be authorized to act as an agent or default surrogate is not reasonably available; or

6. Complying with an individual's direction under subsection D of Section 9 of this act.

B. An agent, default surrogate, or individual with a reasonable belief that the individual is an agent or a default surrogate shall not be subject to civil or criminal liability or to discipline for unprofessional conduct for a health care decision made in a good faith effort to comply with Section 17 of this act.

SECTION 25. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.25 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. A person shall not:

1. Intentionally falsify, in whole or in part, an advance health care directive;

2. For the purpose of frustrating the intent of the individual who created an advance health care directive or with knowledge that doing so is likely to frustrate the intent:

a. intentionally conceal, deface, obliterate, or delete the directive or a revocation of the directive without consent of the individual who created or revoked the directive, or

- b. intentionally withhold knowledge of the existence or revocation of the directive from a responsible health care professional or health care institution providing health care to the individual who created or revoked the directive;

3. Coerce or fraudulently induce an individual to create, revoke, or refrain from creating or revoking an advance health care directive or a part of a directive; or

4. Require or prohibit the creation or revocation of an advance health care directive as a condition for providing health care.

B. An individual who is the subject of conduct prohibited under subsection A of this section, or the individual's estate, shall have a cause of action against a person who violates subsection A of this section for statutory damages of Twenty-five Thousand Dollars (\$25,000.00) or actual damages resulting from the violation, whichever is greater.

C. Subject to subsection D of this section, an individual who makes a health care instruction, or the individual's estate, shall have a cause of action against a health care professional or health care institution that intentionally violates Section 22 of this act for statutory damages of Fifty Thousand Dollars (\$50,000.00) or actual damages resulting from the violation, whichever is greater.

D. A health care professional who is an emergency medical responder shall not be liable under subsection C of this section for a violation of subsection E of Section 22 of this act if:

1. The violation occurs in the course of providing care to an individual experiencing a health condition for which the professional reasonably believes the care was appropriate to avoid imminent loss of life or serious harm to the individual;

2. The failure to comply is consistent with accepted standards of the profession of the professional; and

3. The provision of care does not begin in a health care institution in which the individual resides or was receiving care.

E. In an action under this section, a prevailing plaintiff may recover reasonable attorney fees, court costs, and other reasonable litigation expenses.

F. A cause of action or remedy under this section shall be in addition to any cause of action or remedy under any other law.

SECTION 26. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.26 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. A physical or electronic copy of an advance health care directive, revocation of an advance health care directive, or appointment, designation, or disqualification of a surrogate has the same effect as the original.

B. An individual may create a certified physical copy of an advance health care directive or revocation of an advance health care directive that is in electronic form by affirming under penalty of perjury that the physical copy is a complete and accurate copy of the directive or revocation.

SECTION 27. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.27 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. On petition of an individual, the individual's surrogate, a health care professional or health care institution providing health care to the individual, or a person interested in the welfare of the individual, the court may:

1. Enjoin implementation of a health care decision made by an agent or default surrogate on behalf of the individual, on a finding that the decision is inconsistent with Section 17 or 18 of this act;

2. Enjoin an agent from making a health care decision for the individual, on a finding that the individual's appointment of the agent has been revoked or the agent:

- a. is disqualified under subsection B of Section 8 or under Section 21 of this act,
- b. is unwilling or unable to comply with Section 17 of this act, or
- c. poses a danger to the individual's well-being;

3. Enjoin another individual from acting as a default surrogate, on a finding that the other individual acting as a default surrogate did not comply with Section 12 of this act or the other individual:

- a. is disqualified under Section 14 or 21 of this act,
- b. is unwilling or unable to comply with Section 17 of this act, or
- c. poses a danger to the first individual's well-being,

and authorize another member of the classes identified in subsection B of Section 12 of this act who is able and willing to comply with the provisions of this act to serve as the individual's default surrogate in place of the enjoined surrogate; or

4. Order implementation of a health care decision made:

- a. by and for the individual, or
- b. by an agent or default surrogate who is acting in compliance with the powers and duties of the agent or default surrogate.

B. In this act, advocacy for the provision, withholding, or withdrawal of health care or mental health care to or from an individual is not itself evidence that an agent or default surrogate, or a potential agent or default surrogate, poses a danger to the individual's well-being.

C. A proceeding under this section shall be governed by Sections 2-101 et seq. and 3-101 et seq. of Title 30 of the Oklahoma Statutes.

SECTION 28. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.28 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. This act shall not authorize mercy killing, assisted suicide, or euthanasia, and is subject to the provisions of the Assisted Suicide Prevention Act.

B. This act shall not affect other laws of this state governing treatment for mental illness of an individual involuntarily

committed to a mental health care institution under Section 5-410 et seq. of Title 43A of the Oklahoma Statutes.

C. Death of an individual caused by withholding or withdrawing health care in accordance with this act shall not constitute a suicide or homicide or legally impair or invalidate a policy of insurance or an annuity providing a death benefit, notwithstanding any term of the policy or annuity.

D. This act shall not create a presumption concerning the intention of an individual who has not created an advance health care directive, except as provided in the Hydration and Nutrition for Incompetent Patients Act.

E. An advance health care directive created before, on, or after the effective date of this act shall be interpreted in accordance with the laws of this state, excluding the state's choice of law rules, at the time the directive is implemented.

SECTION 29. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.29 of Title 63, unless there is created a duplication in numbering, reads as follows:

In applying and construing this uniform act, a court shall consider the promotion of uniformity of the law among jurisdictions that enact it, except with regard to provisions of this act that substantively differ from provisions in the act as proposed by the Uniform Law Commission.

SECTION 30. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.30 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. Subject to Section 19 of this act, an advance health care directive created before the effective date of this act shall be valid if it complies with this act or complied at the time of creation with the laws of the state in which it was created.

B. Except as provided in subsection E of Section 28 of this act, this act shall not affect the validity or effect of an act done before the effective date of this act.

C. An individual who assumed authority to act as default surrogate before the effective date of this act may continue to act as default surrogate until the individual for whom the default

surrogate is acting has capacity or the default surrogate is disqualified, whichever occurs first.

SECTION 31. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.31 of Title 63, unless there is created a duplication in numbering, reads as follows:

This act applies to an advance health care directive created before, on, or after the effective date of this act.

SECTION 32. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.32 of Title 63, unless there is created a duplication in numbering, reads as follows:

Upon motion by any party to enjoin the implementation of a health care decision by a surrogate to provide, withhold, or withdraw life-sustaining care, the court shall issue an order requiring that, pending a final decision on the merits and the resolution of any appeal, the individual be provided with health care which, in reasonable medical judgment, is necessary to prevent the individual's death. The court shall not order the provision of such care if doing so would require the denial of the same health care to another individual.

SECTION 33. REPEALER 63 O.S. 2021, Sections 3101.1, 3101.2, 3101.3, 3101.4, 3101.5, 3101.6, 3101.7, 3101.8, 3101.9, 3101.10, 3101.11, as amended by Section 737, Chapter 486, O.S.L. 2025, 3101.12, 3101.13, 3101.14, 3101.15, 3101.16, 3102.4, as amended by Section 15, Chapter 136, O.S.L. 2022, and 3102.5 (63 O.S. Supp. 2025, Sections 3101.11 and 3102.4), are hereby repealed.

SECTION 34. REPEALER Sections 1, 2, 3, as amended by Section 1, Chapter 154, O.S.L. 2023, 4, 5, as amended by Section 2, Chapter 154, O.S.L. 2023, 6, 7, 8, 9, 10, 11, 12, and 13, Chapter 136, O.S.L. 2022 (63 O.S. Supp. 2025, Sections 3111.1, 3111.2, 3111.3, 3111.4, 3111.5, 3111.6, 3111.7, 3111.8, 3111.9, 3111.10, 3111.11, 3111.12, and 3111.13), are hereby repealed.

SECTION 35. This act shall become effective July 1, 2027.

Passed the House of Representatives the 6th day of May, 2026.

Presiding Officer of the House
of Representatives

Passed the Senate the 5th day of May, 2026.

Presiding Officer of the Senate

OFFICE OF THE GOVERNOR

Received by the Office of the Governor this _____

day of _____, 20_____, at _____ o'clock _____ M.

By: _____

Approved by the Governor of the State of Oklahoma this _____

day of _____, 20_____, at _____ o'clock _____ M.

Governor of the State of Oklahoma

OFFICE OF THE SECRETARY OF STATE

Received by the Office of the Secretary of State this _____

day of _____, 20_____, at _____ o'clock _____ M.

By: _____

